

Final Medication Recommendation and Authorisation Addendum

Standalone finality gates, exact instruction requirements, and ownership

Document ID	Version	Effective	Review due
NB-GOV-102	1.0	2026-08-17	2027-08-17

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Owner: Medical Director
Approved by: Fictional Clinical Governance Committee
Classification: Training / synthetic corpus

Document control and retrieval metadata

Field	Value
Canonical title	Final Medication Recommendation and Authorisation Addendum
Document ID	NB-GOV-102
Keywords	final recommendation, authorization, prescription, exact instructions, ownership, clinician review
Authority	Medical Director
Applicability	Training simulations only
Relationship	Standalone extension; existing corpus documents remain unchanged

Retrieval rule

Always retrieve when the system is asked for a final medication recommendation or prepares administration, testing, or follow-up instructions.

1. Definition of final

A recommendation is FINAL_AUTHORIZED only when a complete patient-specific medication order is authorized by a simulated clinician and all deterministic identity, reconciliation, allergy, monitoring, conflict, provenance, claim, citation, and output gates pass.

The model may draft language but cannot create authorization, sign an order, select FINAL_AUTHORIZED, or release its own output.

2. Complete medication recommendation

Section	Required content
Action	CONTINUE, START, ADJUST, HOLD, STOP, NO_CHANGE, or NO_RECOMMENDATION
Medication	Name, formulation, strength, dose/quantity, unit, route, frequency
Timing	Specific timing, food/formulation instruction, start, duration/stop/review
Missed dose	Current medicine-specific authorized wording; never inferred
Rationale	Patient facts plus current supporting evidence
Authorization	Status, clinician, order ID/version, signed timestamp
Warnings	Patient-specific risks, red flags, interactions, contraindications, self-adjustment prohibitions
Monitoring	Tests/observations, why, date/window, booking owner, results owner, escalation
Follow-up	Appointment type, urgency, window, prerequisites, responsible service, outcome route
Uncertainty	Missing/stale data, conflicts, tool failures, evidence grade, limitations
Evidence	Claim-level patient and document citations

3. Ownership and closed loop

Item	Required owner
Medication decision	Authorizing clinician
Interaction/reconciliation	Clinical pharmacist or authorized clinician
Test booking	Diagnostics/booking service
Result review	Named clinician with backup
Patient education	Named nurse/pharmacist/clinician
Appointment booking	Booking service

Item	Required owner
Safety escalation	Duty clinical team
Policy/citation issue	Knowledge governance
Final review	Identified human reviewer

4. Prohibited completion shortcuts

- Do not transform 'continue current medication' into a dose instruction when the current order is incomplete.
- Do not infer once/twice daily from general product knowledge.
- Do not invent missed-dose, food, crushing, splitting, hold, or restart advice.
- Do not treat a planned test as completed or a collected test as reviewed.
- Do not produce a final recommendation merely because the user requested one.
- Do not omit conflicts, uncertainty, tool failures, authorization state, or safety net to make an answer appear complete.

5. Non-final output

When finality gates fail, return NO_RECOMMENDATION or a clearly labelled DRAFT_FOR_CLINICIAN_REVIEW; preserve verified facts; identify exactly what blocks completion; provide the safest next action, owner, and urgency; and do not fill administration fields by inference.

6. Audit record

- Request/user role and patient-binding token.
- Patient-record versions and observation times.
- Order ID/version, authorizer, and signature timestamp.
- Retrieved source IDs/versions/chunks and index version.
- Deterministic gate results, conflicts, tool failures, model/prompt version, validator outcomes, final status, reviewer, and release time.
- Avoid unnecessary identifiable information in external observability.

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